

Does GLP-1 Make You Blind?

A case study in two faults: relative reporting and the causal leap

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The news

Patients are suing the makers of Ozempic, Wegovy, and Mounjaro. They say the drugs caused sudden loss of sight. The condition is called NAION. The blood supply to the nerve behind the eye is cut off, and the sight that is lost usually does not come back. Europe put a warning on the label in 2025. The FDA is still looking. The drug companies say the drugs have not been shown to cause it.

So, does GLP-1 make you blind? The headline number rests on two of the six faults. Take them in order.

Patients Sue GLP-1 Drugmakers Over Risk of Sudden Vision Loss. Wall Street Journal, September 2026.

Fault 1, relative reporting

The first fault is relative reporting. A risk is stated as a multiple, and the multiple sounds far larger than the number of people it happens to.

The story started with a study from a Boston eye hospital in 2024. It found that people on semaglutide, the drug in Ozempic and Wegovy, were four to seven times as likely to get NAION as people on other drugs. Seven times the risk of going blind. That is the number the lawyers use, and it is the number you will hear.

But seven times what? That sentence tells you how much bigger one risk is than another. It does not tell you how big either one is.

The European drug regulator looked at all the evidence in 2025. Here is what it found, counted in people. Take 10,000 people with type 2 diabetes who are not on the drug. In a year, about 1 of them gets NAION. Take 10,000 who are on the drug. In a year, about 2 get it.

That is the whole story. One extra person in ten thousand, per year.

The regulator calls this “very rare.” It is the same fact whether you say “double the risk” or “one in ten thousand.” Only one of those tells you how many people it happens to.

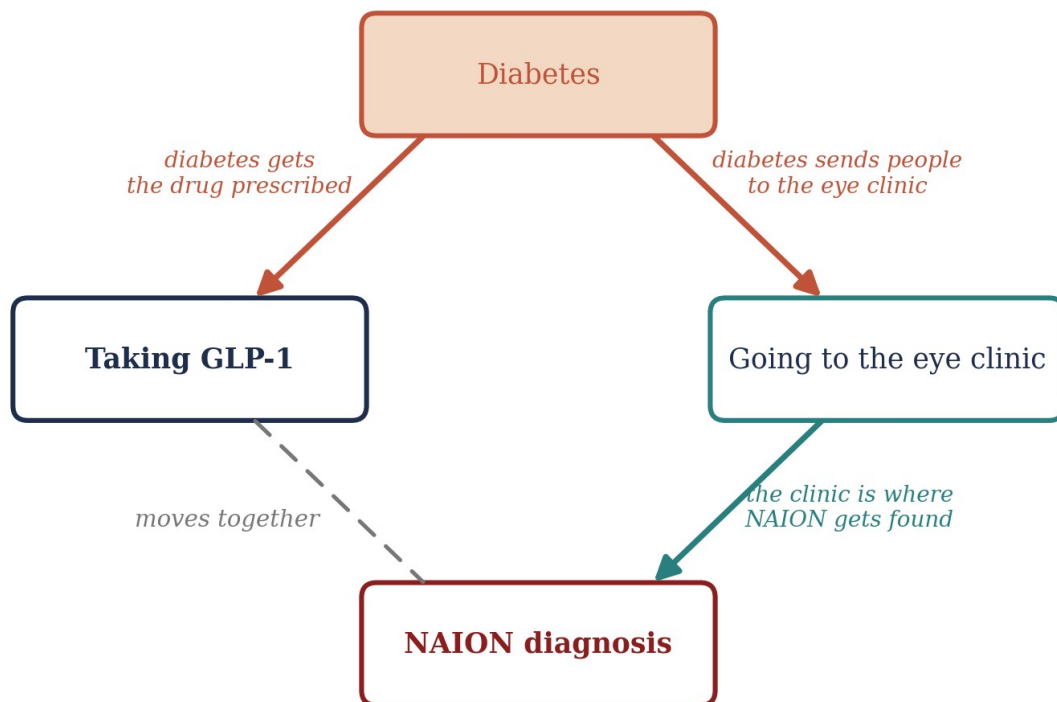
Hathaway JT et al. JAMA Ophthalmology 2024;142:732–739. · European Medicines Agency, 6 June 2025: NAION is a very rare side effect of semaglutide, about one extra case per 10,000 person-years. Confirmed by a Danish and Norwegian study of about 60,000 users (Simonsen E et al., 2024).

Fault 5, the causal leap

The second fault is the causal leap. Two things move together, and an arrow gets drawn between them.

The Boston study was not wrong about what it counted. It counted an unusual group of people. Every patient in it was at a clinic for eye-nerve problems. They were there because something was already wrong with their eyes. In that group NAION is common, and a difference between two groups looks huge.

Now look at what sits behind both columns. Diabetes does two things at once. It gets people prescribed the drug, and it sends people to the eye clinic. The eye clinic is where NAION gets diagnosed. So the drug and the diagnosis travel together whether or not the drug causes the harm. The study did not measure any of that. The authors said themselves that their patients were not typical. The big number got quoted. The warning did not.



Solid arrow: causes. Dashed line: moves together.

The fork. Diabetes leads to the drug and to the eye clinic. Both lead to a NAION diagnosis.

Later studies used the health records of whole countries, where nobody was picked for having eye trouble. They found about two to three times the risk, not seven. And two or three times one in ten thousand is still two or three in ten thousand.

Study population and 36-month rates from Hathaway et al., cited above. National register results: Simonsen E et al., cited above.

So, does it?

It can. Rarely. About one extra person in ten thousand, each year. Whether that is worth it depends on why you are taking the drug, and that is a conversation for you and your doctor.

Two faults in one headline. Fault 1: a ratio with no count behind it. Fault 5: a count taken from people who were already at the eye clinic.

What you can do is walk in with the right question. Not “how many times higher?” but “out of ten thousand people like me, how many?”